BASELINE → TREATMENT → FOLLOW-UP

When checks happen

Columns show order, not equal durations. The first routine 12-week echo falls during the six-cycle preoperative course, around cycle 5 if uninterrupted. Count from first HER2 treatment; a delay, surgery or a change of medicine does not restart the clock. The selected postoperative regimen determines later tests.

Check	Before starting	Cycles 1–4	Around cycle 5 / week 12	Cycle 6 / surgery	Postoperative plan
Blood tests	full blood count (FBC), renal and liver profile; verify unindexed GFR for carboplatin	FBC, renal and liver profile before every cycle.	Continue pre-cycle bloods	Before cycle 6; preoperative tests as directed	Follow the selected postoperative medicine; do not carry forward taxane-day checks
Echo / multigated acquisition (heart pumping) scan (MUGA)	Before the first HER2 treatment	Earlier if symptoms or concern	Echo due at about 12 weeks (cycle 5 if on time)	Review any due echo; earlier if symptoms	Continue on the selected HER2 plan; surgery does not restart the clock
electrocardiogram (ECG) / cardiac markers	ESC: baseline ECG and cardiovascular risk review; baseline troponin/BNP if they will be used for monitoring	Repeat when symptoms, risk or the cardiac plan indicate	ECG does not replace echo	Markers are risk-based, not automatic tumour-marker tests	Follow up abnormal results
Scans / cancer markers	Document breast/nodal disease and staging as indicated	Clinical response; earlier imaging for concern	multidisciplinary team (MDT)-selected response imaging; not automatically tied to the week-12 echo	Reassess before surgery	Breast surveillance; symptom-directed other imaging
dual-energy X-ray absorptiometry (bone density) (DEXA) / bone health	Assess endocrine indication and fracture risk	No routine cycle-by-cycle DEXA	Plan baseline DEXA if aromatase inhibitor (AI) / ovarian suppression indicated	Repeat according to risk, results and endocrine therapy	Continue the agreed bone-health plan

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. National Cancer Control Programme (NCCP) 00789 v3 · official regimen · ESMO 2024 early breast cancer guideline · treatment and follow-up · ESC 2022 cardio-oncology guideline · cardiac surveillance and risk · NICE NG101 · bone health and follow-up · ESMO metastatic breast cancer guideline · 2026 · EMA Phesgo product information

THE WHOLE TREATMENT JOURNEY

TCHP with Phesgo: the whole pathway

HER2-positive neoadjuvant breast cancer

Six 21-day cycles before surgery: Phesgo, docetaxel and carboplatin on Day 1. Surgery and pathology determine the postoperative HER2 medicine and remaining course.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Baseline	Neoadjuvant cycles 1–6	Surgery / pathology	Postoperative treatment	Follow-up
Cancer medicines NCCP 00789	Confirm HER2 status, indication and baseline checks	Phesgo + docetaxel / carboplatin Day 1; six cycles	Surgery and pathology select adjuvant medicines	Selected adjuvant HER2 course, aiming for one year overall	Review outcomes and next plan
Surgery EARLY	Plan after neoadjuvant therapy	Systemic treatment first	Lumpectomy / mastectomy after recovery	Pathology guides the next medicines	Local follow-up
Radiotherapy EARLY	Assess indication	Usually after chemotherapy and surgery	Plan after surgery / recovery	May overlap HER2-only treatment if agreed	Review late effects
Blood tests NCCP 00789	Baseline FBC, renal / liver	FBC, renal and liver profile before every cycle.	Check before changing phase	Use the blood-test schedule for the separately selected postoperative medicine	Active medicines / abnormal results determine follow-up
Echo / heart tests NCCP 00789 · ESC · SMPC	Baseline echo / MUGA; ECG and cardiovascular risk review	Echo every 12 weeks: first around cycle 5; earlier for symptoms	Review any due cardiac test; surgery does not restart the clock	Continue cardiac checks for the selected HER2 medicine	Agree cardiac follow-up when treatment ends
Imaging / markers EARLY · MARKERS	Baseline breast / nodal assessment; staging as indicated	Clinical response and MDT-selected imaging during treatment	Reassess before surgery; pathology informs the next plan	Mammography of remaining breast tissue under the surveillance plan	Other imaging for symptoms / clinical concern; no routine tumour-marker screening
Endocrine / DEXA BONE · EARLY	Check hormone receptors and fracture risk	Discuss the later plan	Baseline DEXA when indicated	Endocrine therapy may overlap HER2 / radiotherapy (RT) if agreed	Bone and endocrine follow-up

What each check is for

1

Blood tests

FBC, renal and liver profile before every cycle.

Check treatment safety.

Ask / act: The treating team records the booking owner, due window, result reviewer and action if overdue.

Sources: NCCP 00789

2

Echo / MUGA

Echo or MUGA before starting and every 12 weeks during treatment, including maintenance. At an uninterrupted 21-day schedule, the first 12-week check is around cycle 5; count elapsed time from the first HER2 dose. A delay or route/phase change does not restart the clock. Cardiac concern may require 4–8-week checks or urgent earlier assessment.

Monitor heart pumping function.

Ask / act: The treating team records the booking owner, due window, result reviewer and action if overdue.

Sources: NCCP 00789

3

Cancer imaging

Baseline breast/nodal assessment and staging as indicated; assess response during neoadjuvant treatment and before surgery using the MDT-selected imaging. After local treatment, arrange mammography of remaining breast tissue; routine computed tomography (CT)/positron emission tomography (PET) or tumour markers are not asymptomatic early-breast recurrence screening.

Assess disease response or surveillance.

Ask / act: The treating team records the booking owner, due window, result reviewer and action if overdue.

Sources: EARLY

4

Bone health

If hormone-receptor-positive disease leads to an aromatase inhibitor or ovarian suppression, assess fracture risk and baseline DEXA when indicated. Repeat according to results, risk and the endocrine plan; DEXA is not a scan at each Phesgo cycle.

Reduce fracture risk when endocrine therapy affects bone.

Ask / act: The treating team records the booking owner, due window, result reviewer and action if overdue.

Sources: BONE

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-10-21 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.15